
Hormone Therapy

For cancers that need specific hormones to grow, hormone therapy can sometimes stop or slow their growth. Hormone therapy is most often given to treat breast and prostate cancer, but may be used for other cancers as well. Learning about how it works and what to expect can help you prepare for treatment and make informed decisions about your care.

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What is hormone therapy?

Hormone therapy is a type of cancer treatment that works by preventing cancer from getting the hormones it needs to grow. Depending on the type of hormone therapy used, it can:

- Stop the body from making the hormone
- Block the hormone from stimulating the cancer cells

Most hormone therapies are drugs taken as oral medicine or injections. Some types of surgery to remove organs that produce hormones can also be a type of hormone therapy.

Hormone therapy is also called **hormonal therapy**, **anti-hormonal therapy**, or **endocrine therapy**. When used as treatment for prostate cancer, it can also be called **androgen deprivation therapy (ADT)** or **testosterone suppression therapy**.

What are hormones?

Hormones are chemicals made by the body that send messages to control how certain types of cells or organs work. For example, sex hormones such as estrogen, testosterone, and progesterone help control sexual development and reproduction. Other hormones include thyroid hormones, cortisol, adrenaline, and insulin.

Different types of hormones help manage:

- Growth and development
- Sexual function
- Reproduction
- Mood and emotions
- Metabolism (how the body turns food into energy)

Major hormone-producing organs or glands include the thyroid, parathyroid glands, adrenal glands, pancreas, ovaries, and testicles. Each one makes specific hormones that affect different parts of the body.

How is hormone therapy used to treat cancer?

Hormone therapy treats cancer by preventing **hormone-sensitive cancer cells** from getting the hormones they need to grow and divide. This results in slower cancer growth and spread, and in some cases, can shrink the tumor.

The goal of hormone therapy depends on the type of cancer and how far it has spread. Hormone therapy can be used in different ways at different times. It might be used:

- Before surgery and/or radiation therapy, to shrink a tumor (**neoadjuvant therapy**).

- After cancer treatments, to reduce the risk that cancer will come back (**adjuvant therapy**).
- For cancer that comes back after treatment (**recurrent cancer** or **recurrence**).
- For cancer that has spread to other parts of the body (**metastatic cancer**).

Hormone therapy may also be used to help prevent or manage cancer side effects. Relieving side effects to help improve a person's quality of life is an important part of cancer care and treatment. This is called [palliative care](#)¹.

[Watch this video on YouTube](#) ²

Which cancers can be treated with hormone therapy?

Hormone therapy is mostly used to treat certain kinds of breast cancer and prostate cancer that depend on sex hormones to grow. Other cancers that might be treated with hormone therapy include thyroid, adrenal, endometrial (uterine), and some types of ovarian cancer.

Is hormone therapy the same as hormone replacement?

Hormone therapy for cancer treatment is not the same as **menopausal hormone therapy (MHT)** or **hormone replacement therapy (HRT)**. These treatments are used to manage the symptoms of menopause.

It is also different from hormonal therapy given after the surgical removal of the thyroid gland ([thyroidectomy](#)³) to treat thyroid cancer. This is called **thyroid hormone therapy**. In this case, it is given to replace the natural thyroid hormone that the body needs, but is no longer able to make after the thyroid gland is removed.

Hormone therapy for cancer treatment is also different from hormone therapy used by transgender people (**gender-affirming hormone therapy or GAHT**). If you are a transgender person with cancer and hormone therapy is a treatment option, please talk to your cancer care team for more information about how the treatment affects your situation.

How is hormone therapy different from other cancer treatments?

Hormone therapy is considered **systemic treatment** (like chemotherapy and other drug treatments) because the hormones it targets circulate throughout the body. The drugs used in hormone therapy travel throughout the body to find, target, and act on the hormones.

This makes hormone therapy different from **local treatments** like surgery and radiation therapy, which only affect only a certain part of body.

Even when hormone therapy involves surgery to remove hormone-making organs, it is still considered systemic because it blocks the production of the hormone throughout the entire body.

Types of hormone therapy

There are several different types of hormone therapy. Most are drugs designed to block or lower the amount of certain hormones in the body. Some forms involve surgery to remove hormone-producing organs. Options for hormone therapy depend on the type of cancer you have.

Hormone therapy for breast cancer

Some breast cancers need the hormones **estrogen** and/or **progesterone** to grow. These are called **hormone receptor-positive (HR-positive) breast cancers** because the cancer cells have receptors on their surface that these hormones bind to.

Blocking the hormones or lowering the amount of estrogen your body makes can help treat metastatic cancer or prevent a recurrence.

Examples of [hormone therapy for breast cancer](#)⁴ include:

- **Aromatase inhibitors (AIs)**, such as anastrozole (Arimidex), exemestane (Aromasin), and letrozole (Femara)
- **Selective estrogen receptor modulators (SERMs)**, such as tamoxifen and toremifene (Fareston)
- **Selective estrogen receptor degraders (SERDs)**, such as fulvestrant (Faslodex), elacestrant (Orserdu), and imlunestrant (Inluriyo)

- **Luteinizing hormone-releasing hormone (LHRH) agonists**, such as goserelin (Zoladex) and leuprolide (Lupron)
- Surgery to remove the ovaries (**oophorectomy**)

Hormone therapy for prostate cancer

The growth of prostate cancer is usually driven by hormones called **androgens**. The most common androgen is testosterone.

Lowering levels of androgen can help slow the growth of prostate cancer. This is called **androgen deprivation therapy (ADT)**.

Examples of [hormone therapy for prostate cancer](#)⁵ include:

- **Anti-androgens**, such as bicalutamide (Casodex), flutamide (Eulexin), and nilutamide
- **Androgen receptor pathway inhibitors**: such as apalutamide (Erleada), enzalutamide (Xtandi), and darolutamide (Nubega)
- **CYP17 inhibitors**, such as abiraterone (Zytiga) and ketoconazole (Nizoral)
- **Luteinizing hormone-releasing hormone (LHRH) agonists** such as goserelin (Zoladex), leuprolide (Lupron), and triptorelin (Trelstar)
- **Luteinizing hormone-releasing hormone (LHRH) antagonists**, such as degarelix (Firmagon) and relugolix (Orgovyx)
- Surgery to remove the testicles (**orchiectomy** or **surgical castration**)

Hormone therapy for endometrial (lining of the uterus or womb) cancer

Hormone therapy can slow the growth of certain types of endometrial (uterine) cancer cells that are sensitive to **estrogen** and **progesterone**.

Examples of [hormone therapy for endometrial cancer](#)⁶ include:

- **Progestins**, such as medroxyprogesterone acetate (Provera) or megestrol acetate (Megace)
- **Selective estrogen receptor degraders (SERDs)**, such as fulvestrant (Faslodex)

- **Selective estrogen receptor modulators (SERMs)**, such as tamoxifen
- **Aromatase inhibitors (AIs)**, such as letrozole (Femara), anastrozole (Arimidex), and exemestane (Aromasin)

Hormone therapy for adrenal cancer

An adrenal gland tumor may produce excess hormones such as cortisol, aldosterone, norepinephrine, and epinephrine. Less often, they can make excess sex hormones like testosterone and estrogen. Your doctor may prescribe various medications to control the levels of these hormones before, during, or after other treatments.

Examples of [hormone therapy for adrenal cancer](#)⁷ include:

- **Adrenolytics**, such as mitotane
- **Anti-androgens (androgen receptor antagonists)**, such as bicalutamide (Casodex)
- **Diuretics**, such as amiloride and spironolactone
- **Aldosterone receptor antagonists**, such as eplerenone
- **5-alpha-reductase inhibitors**, such as finasteride
- **Glucocorticoid receptor antagonists**, such as mifepristone
- **CYP17 inhibitors**, such as ketoconazole (Nizoral)
- **Steroid 11-beta-monooxygenase inhibitors**, such as metyrapone
- **Anesthetic agents**, such as etomidate
- **Selective estrogen receptor modulators (SERMs)**, such as tamoxifen and raloxifene

Hormone therapy for ovarian cancer

Some types of ovarian cancer, such as **stromal tumors**, can be treated with hormone therapy. These treatments work in different ways. Some lower the amount of **estrogen** the body makes. Others block cancer cells from using estrogen already in the body. Lowering or blocking estrogen can help slow the growth of cancers that need it to grow.

Examples of [hormone therapy for ovarian cancer](#)⁸ include:

- **Luteinizing hormone-releasing hormone (LHRH) agonists**, such as goserelin (Zoladex) and leuprolide (Lupron)

- **Selective estrogen receptor modulators (SERMs)**, such as tamoxifen
- **Aromatase inhibitors (AIs)**, such as letrozole (Femara), anastrozole (Arimidex), and exemestane (Aromasin)

When is hormone therapy recommended?

Hormone therapy is only used for certain cancers that can be affected by changing the levels of specific hormones. Your cancer care team will consider many factors before recommending hormone therapy, including:

- The type and subtype of cancer
- The stage of the cancer (where it is, how big it is, and whether it's spread to other parts of the body)
- Results of other tests on the tumor, such as biomarkers
- Your age
- Your overall health and current medications
- Other serious health problems (such as heart, liver, or kidney diseases)
- Goal of treatment
- Other cancer treatments you have had in the past or are currently receiving
- The risk of cancer returning or how long it has been before the cancer returned
- Whether or not you have gone through menopause

What is it like to get hormone therapy?

Getting hormone therapy can be very different depending on the type you need. Some are oral medicines you take at home, while others are injections you might get at your doctor's office or cancer center. Some surgeries are also a form of hormone therapy.

Oral hormone therapy

Many types of hormone therapy are drugs that are taken by mouth. In these cases, you swallow the pill, capsule, or liquid just like other medicines. These are usually taken at home.

How often they're taken depends on the drug and the type of cancer being treated. Because of this, it's important to be sure you know exactly how to take it and follow instructions exactly.

Injectable hormone therapy

Some types of hormone therapy are given as injections. They can be:

- **Intramuscular (or IM) injections** given in the muscle of the arm, leg, or hip.
- **Subcutaneous (SC or sub-Q) injections**, given just under the skin of the abdomen (belly).

How often they're given depends on the drug and type of cancer being treated. At first, the injections might be given in your treatment center or doctor's office. Sometimes you will be taught how to give your own injections or a caregiver can be taught to give them to you. Then you can take the injections at home.

Surgery to remove hormone-making organs

Some types of surgery can also be forms of hormone therapy:

- **Orchiectomy** (surgery to remove the testicles, the body's main source of testosterone) can be an option for some men with prostate cancer who need hormone therapy as part of their treatment.
- **Oophorectomy** (surgery to remove the ovaries, the body's main source of estrogen and progesterone) can be an option for some women with breast cancer.

Side effects from these surgeries tend to be like those from drugs that lower hormone levels in the body. An advantage of this type of hormone therapy is that it is done all at once, and it doesn't require long-term treatment with medicines. A possible downside is that it is permanent, so once it's done, it can't be reversed.

Hormone therapy drug safety

Some hormone therapy drugs are considered hazardous and have safety precautions that should be followed. Often, these are similar to the [precautions used for chemotherapy safety](#)⁹.

Talk to your cancer care team about any special precautions that might be needed to protect yourself and others while you are taking hormone therapy.

What are the side effects of hormone therapy?

The side effects of hormone therapy depend on the type and dose of hormone therapy, as well as your overall health and whether you are getting other types of cancer treatment.

Hormone Therapy Side Effects

While hormone therapy can be an effective type of cancer treatment, it often causes side effects. It's important to know about possible side effects when making treatment decisions. Learn more.

Hyperlinks

1. www.cancer.org/cancer/supportive-care/palliative-care.html
2. www.youtube.com/embed/c2TrhSnCVnQ?origin=http%3A%2F%2Flocalhost%3A4502&hl=en&enablejsapi=1
3. www.cancer.org/cancer/types/thyroid-cancer/treating/surgery.html
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Hormone Therapy Side Effects

While hormone therapy can be an effective type of cancer treatment, it often causes side effects. It's important to know about possible side effects when making treatment decisions. It's also important to balance the benefits and risks of any treatment as they relate to your goals and quality of life.

[Why does hormone therapy cause side effects?](#)

[What do I need to know about side effects?](#)

[How long do side effects from hormone therapy last?](#)

[What are common side effects of hormone therapy?](#)

[How can I help manage these side effects?](#)

Why does hormone therapy cause side effects?

Hormones are chemicals made by the body that control the activity of certain cells or organs. They move through the bloodstream. Changes to the amount of a hormone in your body can affect what that hormone controls and can cause specific side effects. Some hormones affect body functions, so hormone therapy can cause many different side effects.

What do I need to know about side effects?

- Every person doesn't get every possible side effect, and some people get few or none.
- The severity of side effects (how bad they are) varies from person to person.
- Your doctor may give you medicines to help prevent certain side effects before they happen.
- Some hormone therapy causes long-term side effects, like heart or vision problems. Still, many people have no long-term problems from hormone therapy.

While side effects can be unpleasant, they must be weighed against the need to kill the cancer cells. Talk to your cancer care team about:

- Which side effects are most common with your hormone therapy
- How long they might last
- How severe they might be
- What steps to take if side effects develop, including when and who to call

How long do side effects from hormone therapy last?

Some side effects are **short-term**. They go away fairly quickly after hormone therapy is started (or once it stops) or they might last for as long as you are in treatment.

Examples include hot flashes or sexual changes. But some might be **long-term** or **chronic side effects**. They start during treatment but continue even after treatment is over, sometimes lasting for months or years. Examples include vision changes or heart problems.

Certain types of hormone therapy sometimes cause **late side effects** after treatment ends. These side effects can happen long after cancer treatment, such as bone loss (**osteoporosis**) or increased risk of heart disease.

People often become discouraged about how long their treatment lasts or the side effects they have. If you feel this way, **do not stop taking the hormone therapy on your own**. Talk to your cancer care team. They may be able to change your medicine or treatment schedule. Or they may suggest ways to reduce any pain and discomfort you have.

What are common side effects of hormone therapy?

Each person's side effects can be different. The type of side effects that you may experience depend highly on the type and dose of hormone therapy, as well as your overall health. Side effects may be worse if you are receiving other cancer treatments, such as chemotherapy (chemo) or radiation therapy. Talk to your cancer care team and ask any questions you have about hormone therapy.

Effects of hormone therapy for prostate cancer

Hormone therapy for prostate cancer may cause:

- [Hot flashes or night sweats](#)¹
- [Decreased sexual desire](#)²
- [Erectile dysfunction \(trouble getting an erection\)](#)³
- Bone loss and a higher risk for fractures
- Bone pain (especially when some types of hormone therapy are first started)
- [Infertility](#)⁴
- [Fatigue](#)⁵
- [Nausea](#)⁶
- [Diarrhea](#)⁷
- [Weight gain](#)⁸ (especially around the belly) with decreased muscle mass
- Mood changes
- [Memory problems](#)⁹
- Breast swelling (gynecomastia)
- Increased risk of other health problems, such as heart disease

Learn more in [Hormone Therapy for Prostate Cancer](#)¹⁰.

Effects of hormone therapy for breast, endometrial, or ovarian cancer

Hormone therapy for breast, endometrial, or ovarian cancer may cause the following symptoms, many of which are also symptoms of menopause:

- [Hot flashes or night sweats](#)¹¹
- [Vaginal discharge, dryness, or irritation](#)¹²
- Vaginal bleeding, that is not related to menstrual cycle
- Breast tenderness
- Menstrual changes (if still having periods)
- [Decreased sexual desire](#)¹³
- [Infertility](#)¹⁴
- [Fatigue](#)¹⁵
- [Nausea](#)¹⁶
- [Diarrhea](#)¹⁷
- Pain in muscles and joints

- [Pain during vaginal sex](#)¹⁸
- Mood changes
- Bone loss and a higher risk for fractures (osteoporosis)
- [Weight gain](#)¹⁹ (especially around the belly) with decreased muscle mass
- Higher risk of other types of cancer, [blood clots](#)²⁰, and cataracts

Men with breast cancer who are getting hormone therapy can also experience many of these same side effects, along with [erectile dysfunction](#)²¹.

Learn more in:

- [Hormone Therapy for Breast Cancer](#)²²
- [Hormone Therapy for Breast Cancer in Men](#)²³
- [Hormone Therapy for Endometrial Cancer](#)²⁴
- [Hormone Therapy for Ovarian Cancer](#)²⁵

Effects of hormone therapy for adrenal cancer

Hormone therapy for adrenal cancer involves the use of several different types of medications depending on the specific hormone produced by the tumor. The goal of this treatment is to control the levels of these hormones. Side effects vary across the different medications and hormones. Your doctor will tell you more about what to expect in your case based on the specific type of adrenal tumor you have.

Learn more in [Hormone Therapy for Adrenal Cancer](#)²⁶.

How can I help manage these side effects?

Before hormone therapy begins, talk with your cancer care team about what side effects could happen and how they can be managed. Ask questions about anything that is unclear to you. This will help you feel more prepared.

Let your cancer care team know about any new or worsening side effects or medical problems as soon as possible. This is important even if you don't think your symptoms are serious or related to hormone therapy. Tracking side effects using a calendar or journal to note when they happen and how intense they are can also be helpful to share with your cancer care team.

Take these steps to promote your overall health and help you tolerate hormone therapy better:

- Get adequate sleep
- Be physically active
- Exercise regularly (especially strength training)
- Eat a healthy diet
- Avoid tobacco
- Limit alcohol
- Spend time with friends and family
- Seek emotional support

Hyperlinks

1. www.cancer.org/cancer/managing-cancer/side-effects/hair-skin-nails/hot-flashes-and-sweating.html
2. www.cancer.org/cancer/managing-cancer/side-effects/sexual-side-effects/treatment-and-desire-and-response.html
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